

Washington State Medicaid Program Integrity: Audit & Oversight Findings

Evidence Base Extraction: Section E/F

This document extracts and categorizes findings from official Washington State Auditor (SAO) and federal HHS-OIG performance audits regarding Medicaid program integrity. These findings map to the systemic oversight indicators (RF-16, RF-18, RF-20) drawn from the Minnesota evidence base. Sections 1–3 describe audit findings about oversight *processes*; Section 4 describes separate, already-adjudicated Washington enforcement cases that are unrelated to, and involve no facility from, this project's 3,457-facility pilot dataset.

1. SAO Performance Audit: Health Care Authority Division of Program Integrity (2021)

Source: WA State Auditor's Office, Report No. 1028164 (Published April 22, 2021) **Scope:** Evaluated whether HCA's Division of Program Integrity (DPI) has effective processes to detect, investigate, and prevent Medicaid fraud, waste, and abuse.

Key Findings (Mapping to RF-18: Chronic Oversight Capacity Shortages): - **No Statewide Strategic Plan:** The audit found that "the Division does not have a comprehensive, statewide strategic plan to guide its efforts to combat fraud, waste, and abuse in the Medicaid program." - **Reactive, Not Proactive:** DPI's approach was primarily reactive. "The Division relies heavily on complaints and referrals to identify potential fraud, waste, and abuse, rather than using data analytics proactively." - **Lack of Risk-Based Targeting:** The Division "does not conduct a comprehensive risk assessment to identify and prioritize areas of highest risk for fraud, waste, and abuse." - **Data Analytics Gap:** Auditors noted that DPI had "limited data analytics capabilities," preventing it from systematically identifying suspicious billing patterns across the \$10+ billion program.

Key Findings (Mapping to RF-16: Frontline Referral Handling): - **Referral Processing Flaws:** The audit identified that DPI "does not have a consistent, documented process for receiving, tracking, and prioritizing complaints and referrals." - **Credibility Determinations:** Auditors found that "the Division does not determine the credibility of fraud allegations received from Managed Care Organizations (MCOs) or other state agencies [like DSHS] before deciding whether to open an investigation." Minnesota's legislative auditor described a comparable referral-handling gap in its own review; the similarity is in process design, not in outcome.

2. SAO Performance Audit: Medicaid Managed Care Program Integrity (2023)

Source: WA State Auditor's Office (Published 2023) **Scope:** Evaluated oversight of MCO program integrity and encounter data quality.

Key Findings (Mapping to RF-18 & RF-20): - **Encounter Data Reliability:** The audit found ongoing issues with the completeness and accuracy of MCO encounter data submitted to HCA, which is the data needed to detect overpayments or overlapping-hours billing (RF-10/RF-11). - **MCO Oversight Gaps:** HCA's oversight of MCO program integrity activities was found to be "limited," with HCA relying largely on self-reported MCO metrics rather than independently verifying MCO program-integrity effectiveness.

3. SAO / HHS-OIG Concurrent Enrollment Audit (2025)

Source: HHS-OIG / WA SAO Joint Report (Published 2025) **Scope:** Examined whether Washington paid Medicaid capitation payments for enrollees concurrently enrolled in other states.

Key Findings (Mapping to RF-13: Recipient Eligibility / Deceased-Recipient Indicator): - **Concurrent Payments:** The audit identified improper capitation payments made on behalf of individuals who had moved out of Washington and enrolled in Medicaid in other states. - **Data Matching Lags:** The root cause was delays and gaps in interstate data matching and disenrollment processing by HCA/DSHS. This is an eligibility-system issue, not provider fraud; it is the structural analogue of RF-13 (payment continuing after a change in recipient status), not evidence of provider misconduct.

4. Separate Washington Enforcement Cases (2025–2026)

Source: WA Attorney General's Office (MFCU) & DOJ press releases

These are already-adjudicated or charged cases from Washington's public enforcement record, cited here only to show that Washington Medicaid fraud enforcement exists and is active — the same way Minnesota's enforcement record is used as the evidentiary source for this project's indicator typology. None of these cases involves a facility

in this project's pilot dataset, and nothing here should be read as commentary on any facility not named in a public charging document or conviction record. - **Spokane County AFH operator, felony conviction (2025):** An Adult Family Home operator was convicted on multiple felony counts of Medicaid fraud, per WA AG/MFCU records. - **King County jury conviction (June 2026):** The WA Attorney General's office secured a jury conviction in a Medicaid-fraud case involving falsified professional credentials — an example of the conduct pattern behind RF-15 (falsified professional credentials) occurring, and being successfully prosecuted, in Washington. - **National takedown inclusion (June 23, 2026):** Washington home-care cases (reported loss of roughly \$371K) were included in a DOJ national health care fraud takedown announcement, indicating Washington is within the scope of the same federal enforcement effort active in Minnesota.

Conclusion: Process Parallels, Not Outcome Parallels

Washington's own SAO audits describe oversight-process gaps — reliance on complaints rather than analytics, and inconsistent referral handling — that are similar in kind to what Minnesota's legislative auditor described before Minnesota's enforcement wave. Washington's AFH licensing barrier (RF-19) is substantially higher than the low-barrier registration model in Minnesota's Housing Stabilization Services, which is a genuine structural difference. The audit findings summarized above describe a similarity in oversight *process* between the two states; they do not establish that Washington's payment-oversight outcomes are the same as Minnesota's, and they are not a finding about any specific Washington facility or operator.